

GLP-1/GIP Agonist — Subcutaneous Injection Instructions

PROVISIONAL CLINICAL CONTENT — This protocol has been drafted using current evidence-based parameters and is operationally usable upon Provisional approval. Full clinical review and final approval by the Covering Physician are scheduled within 180 days. If you have any questions about the technique described in this document, contact your Bloom Metabolics care team via the patient portal before injecting.

Protocol ID: Protocol-002

Medication: Semaglutide / Tirzepatide (GLP-1/GIP Agonist)

Route: Subcutaneous (SubQ)

Version: 0.1

Effective Date: TBD (pending approval)

Reviewed by: Pending Covering Physician Review

Before You Begin

Gather the following supplies before starting:

- & Medication vial (check expiration date — do not use if expired)
- & Insulin syringe: 29-gauge to 31-gauge, 5/16" (8mm) or 1/2" (12.7mm), 0.5mL or 1mL barrel
- & Alcohol swabs (2 — one for the vial top, one for the injection site)
- & Sterile gauze pad
- & FDA-cleared sharps container
- & Clean, flat surface with good lighting

Before every injection:

1. Wash your hands thoroughly with soap and water for at least 20 seconds
2. Inspect the medication — the solution should be clear and colorless. If the solution is cloudy, discolored, or contains particles, do not use it. Contact your care team.
3. Check the expiration date on the vial
4. If the medication has been stored in the refrigerator, allow it to reach room temperature for 15–30 minutes before injecting. Cold medication may cause more discomfort at the injection site.

If something does not look right — the vial is damaged, the solution looks unusual, or the syringe packaging is open — do not inject. Contact your care team.

Identifying Your Syringe

This protocol uses an insulin syringe. This is a small syringe with a fixed (non-removable), very thin needle.

Feature	Insulin Syringe (USE THIS)	Intramuscular Syringe (DO NOT USE)
Barrel size	0.5mL or 1mL	1mL or 3mL
Needle	Fixed (non-removable), 29g–31g, short (5/16" or 1/2")	Detachable, 23g–25g, longer (1 inch)
Needle thickness	Very thin and flexible	Thicker and sturdier
Typical use	Subcutaneous injections (into fat tissue)	Intramuscular injections (into muscle)

If your shipment contains intramuscular syringes (the longer ones), do not use them for this medication. Contact your care team immediately if you inject the medication too deep (into muscle rather than subcutaneous tissue).

Injection Sites

Recommended Sites

Abdomen (Preferred)

The fatty tissue of the abdomen, at least 2 inches (5 cm) away from the navel, bruises, and stretch marks. The abdomen provides the most consistent results.

How to locate: Place two fingers on either side of your navel. The space between your fingers and extends outward. Stay below the ribcage and above the pelvic abdominal region using a clock pattern.

Anterior / Lateral Thigh

The front or outer side of the thigh, in the middle third.

How to locate: Divide your thigh into thirds (top, middle, bottom). The middle portion of the middle third.

Acceptable Sites

Posterior Upper Arm

The back of the upper arm, between the shoulder and elbow. Have someone else inject you or another person for proper technique because it is difficult to see and feel the site.

Site Rotation Schedule

Rotate injection sites weekly. Do not inject into the same site or the same area within the same anatomical site or a different site for 4 weeks.

Example 4-week rotation:

Week	Site	Side / Location	Date	Notes
1	Abdomen	Right of navel		
2	Thigh	Left, outer middle		
3	Abdomen	Left of navel		
4	Thigh	Right, outer middle		

Fill in the date and any observations (e.g., bruising, pain) in the notes column. Bring this tracker to your follow-up appointments.

Step-by-Step Injection Technique

Drawing Medication

1. Clean the top of the medication vial with an alcohol swab. Allow it to air dry.
2. Remove the insulin syringe from its packaging. Do not touch the needle.
3. Pull back the plunger to draw air into the syringe equal to the volume of your prescribed dose.
4. Insert the needle into the vial through the rubber stopper.
5. Inject the air into the vial.
6. Invert the vial and slowly pull back the plunger to draw your prescribed dose into the syringe.
7. Check for air bubbles. If present, tap the syringe gently with your finger to move bubbles to the top, then push the plunger slightly to expel the air. Re-check the volume.
8. Remove the needle from the vial.

Injecting

1. Clean the injection site with a fresh alcohol swab using a circular motion from center outward. Allow to air dry completely.
2. Pinch a fold of skin at the injection site with your non-dominant hand. Lift the subcutaneous tissue away from the underlying muscle. Hold the pinch throughout the injection.
3. Hold the syringe like a pen or dart with your dominant hand.
4. Insert the needle at a 90-degree angle into the pinched skin fold in one smooth, quick motion. Insert the needle to its full depth.
 - For very lean patients with minimal subcutaneous tissue: A 45-degree angle may be more appropriate to ensure the medication is deposited in subcutaneous tissue rather than muscle. Discuss with your care team if you are unsure which angle to use.
1. Inject the medication slowly and steadily over 5–10 seconds.
2. After injecting the full dose, wait 5 seconds before withdrawing the needle.
3. Withdraw the needle in one smooth motion at the same angle it was inserted.
4. Release the skin pinch.
5. Apply gentle pressure with a sterile gauze pad. Do not rub the injection site.

After Injection

1. Immediately dispose of the used syringe in your sharps container. Do not recap the needle.
2. Record the injection in your rotation tracker (site, side/location, date, and any observations).

What to Expect After Injection

Normal findings (not cause for concern):

- A small drop of blood at the injection site
- Slight redness or mild itching at the injection site that resolves within hours
- A small, firm lump under the skin that resolves within a few days
- Mild nausea, especially during dose titration — this is a common medication side effect, not an injection technique issue

Abnormal findings (contact your care team):

- Injection site redness, swelling, or pain that worsens or persists beyond 3 days
- Drainage or pus from the injection site
- Fever following injection
- Severe or persistent nausea, vomiting, or abdominal pain

- Allergic reaction symptoms (hives, swelling of face/lips/tongue, difficulty breathing)
- Signs of pancreatitis: severe abdominal pain radiating to the back, nausea, vomiting — seek emergency care

Troubleshooting

I saw blood after removing the needle.

A small drop of blood at the injection site is normal. Press gently with a clean gauze pad for 10–15 seconds. Do not rub the area.

The needle bent during injection.

Do not attempt to straighten or reinsert a bent needle. Discard the entire syringe in your sharps container and prepare a new dose. Ensure you are pinching the skin firmly and inserting smoothly.

I missed my scheduled injection day.

If less than 5 days have passed since your missed dose, administer it as soon as possible and return to your regular schedule. If more than 5 days have passed, skip the missed dose and take your next dose on the regularly scheduled day. Contact your care team if you are unsure.

The injection site is red, swollen, or painful 24+ hours later.

Mild injection site reactions (redness, slight swelling, itching) are common and typically resolve within a few days. Apply a cool compress if needed. If the reaction is severe, spreading, or accompanied by fever, contact your care team.

I am not sure I injected the full dose.

Check if the plunger is fully depressed in the syringe. If so, you received the full dose. Do not attempt a second injection. Note this at your next check-in with your care team.

I am experiencing nausea after my injection.

Nausea is a common side effect, especially during dose titration. Eat smaller, more frequent meals. Avoid fatty or greasy foods. Stay hydrated. If nausea is severe or persistent, contact your care team — your dose may need adjustment.

When to Contact Your Care Team

Contact your Bloom Metabolics care team via the OptiMantra patient portal if you experience any of the following:

- Injection site signs of infection (increasing redness, warmth, swelling, drainage, fever)
- Severe or persistent nausea, vomiting, or diarrhea
- Severe abdominal pain (especially radiating to the back — may indicate pancreatitis)
- Allergic reaction symptoms
- Uncertainty about your technique — contact us BEFORE injecting
- Questions about dose titration schedule
- Any situation not covered by this troubleshooting guide

Emergency: If you are experiencing a medical emergency — difficulty breathing, severe allergic reaction, symptoms of pancreatitis, loss of consciousness — call 911 immediately. Do not delay emergency care to contact your clinic.

Storage and Disposal

Medication Storage

- Refrigerate at 36–46°F (2–8°C)
- Do not freeze — frozen medication must be discarded
- If needed at room temperature, use within the timeframe specified by your compounding pharmacy (check the label)
- Protect from light — store in the original packaging
- Do not use if the solution is cloudy, discolored, or contains visible particles
- Do not use past the expiration date on the vial

Sharps Disposal

- Dispose of all used syringes immediately in an FDA-cleared sharps container
- Do not recap needles
- When the sharps container is three-quarters full, seal it and follow your local community disposal guidelines
- Many pharmacies accept sharps containers for disposal

Unused Medication

- Do not flush medication down the toilet
- Return unused or expired medication to your pharmacy or follow local medication disposal guidelines

Acknowledgment

Before your first self-injection, you will acknowledge receipt and understanding of these instructions in the OptiMantra patient portal. The acknowledgment statement reads:

"I have reviewed the injection instructions for semaglutide/tirzepatide. I understand the technique, the injection sites, the rotation schedule, and the signs of complications. I will contact my Bloom Metabolics care team via the patient portal with any questions before injecting."

If you have questions about any part of these instructions, contact your care team BEFORE completing the acknowledgment and BEFORE injecting.

Clinical References

- FDA Prescribing Information: Semaglutide — Wegovy (NDA 215256) and Ozempic (NDA 209637). Current FDA labeling at time of protocol drafting.
- FDA Prescribing Information: Tirzepatide — Mounjaro (NDA 215866) and Zepbound (NDA 215919). Current FDA labeling at time of protocol drafting.
- Obesity Medicine Association: Clinical Practice Statement on Pharmacotherapy for Obesity (2024).
- Endocrine Society Clinical Practice Guidelines.
- CDC Guidelines: Injection Practices — Subcutaneous Injections (current).

This protocol is governed by SOP-004 (Patient Self-Administration Injection Education).

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